

Cardiology Consultation Note

Cardiology Department — Encounter Date: 26 Sep 2026

Patient

Patient name	Hassan Raza
MRN	MRN-77190
Date of birth	02 Nov 1965
Consulting physician	Dr. N. Fatima, Cardiology
Encounter date	26 Sep 2026
Ward / bed	Cardiology — Bed 1A

History of Present Illness

Mr. Raza is a 60-year-old male presenting with intermittent exertional chest pain over the past two weeks, described as a dull pressure across the central chest, radiating to the left arm, brought on by climbing stairs and relieved by rest within 5–10 minutes. No associated diaphoresis, nausea, or syncope. No prior cardiac history. Risk factors include hypertension (on amlodipine) and a 15 pack-year smoking history, quit 4 years ago.

Examination & Investigations

Blood pressure 132/84 mmHg, heart rate 78 bpm regular, no murmurs on auscultation, no peripheral edema. Resting ECG shows normal sinus rhythm with no acute ST-T changes. Troponin I within normal limits on serial testing. Exercise stress test shows reproducible ST depression at moderate workload, consistent with inducible ischemia.

Assessment

Stable angina, NYHA class II, in the setting of hypertension and prior smoking history. Suggested diagnosis code: ICD-10 I20.8 (Other forms of angina pectoris).

Plan

1. Start atorvastatin 40mg nightly.
2. Start aspirin 75mg daily.
3. Continue amlodipine for blood pressure control; consider adding metoprolol pending renal function review — dosing above the typical starting range should be confirmed with prescriber given this patient's renal function.
4. Outpatient cardiology follow-up in 2 weeks to reassess symptoms and titrate therapy.
5. Patient counselled on recognizing worsening symptoms and when to seek emergency care.